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YEAR LEVEL AND SECTION: BSN - 3I

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DISEASE	HYPOTHYROIDISM	HYPERTHYROIDISM
DESCRIPTION	A condition where the thyroid gland is underactive and does not produce enough thyroid hormone (T3 and T4) to meet the body's needs.	A condition where the thyroid gland is overactive and produces excessive amounts of thyroid hormones, accelerating the body's metabolism.
CAUSES	• Hashimoto's thyroiditis • Iodine deficiency • Surgical removal of the thyroid • Radiation therapy	• Graves' disease • Toxic multinodular goiter • Thyroiditis • Excessive iodine intake
SIGNS / SYMPTOMS	• Fatigue and weight gain • Cold intolerance • Bradycardia • Dry skin and myxedema (facial puffiness)	• Unexplained weight loss • Heat intolerance • Tachycardia and palpitations • Exophthalmos (bulging eyes)
TREATMENT / MANAGEMENT	• Hormone replacement therapy (Levothyroxine) • Routine TSH monitoring • High-fiber, low-calorie diet	• Antithyroid medications (Methimazole, PTU) • Radioactive Iodine (RAI) therapy • Surgical removal (Thyroidectomy)

DISEASE	HYPOPARATHYROIDISM	HYPERPARATHYROIDISM
DESCRIPTION	Decreased secretion of Parathyroid Hormone (PTH), leading to low serum calcium levels and high phosphate levels.	Excessive secretion of PTH, resulting in high serum calcium (hypercalcemia) and low phosphate levels.
CAUSES	• Accidental damage or removal during neck surgery • Autoimmune destruction • Congenital gland absence	• Primary: Parathyroid adenoma (benign tumor) • Secondary: Chronic kidney disease or Vitamin D deficiency
SIGNS / SYMPTOMS	• Tetany and severe muscle spasms • Positive Chvostek's and Trousseau's signs • Numbness and tingling (circumoral and extremities)	• Bone pain and pathological fractures • Kidney stones (nephrolithiasis) • Fatigue, weakness, and altered mental status
TREATMENT / MANAGEMENT	• IV or oral Calcium and Vitamin D supplementation • High-calcium, low-phosphorus diet • Strict seizure precautions	• Surgical removal of affected glands (Parathyroidectomy) • Aggressive IV hydration to clear calcium • Bisphosphonates

DISEASE	DIABETES MELLITUS
DESCRIPTION	A chronic metabolic disorder characterized by hyperglycemia resulting from defects in insulin secretion, insulin action, or both.
CAUSES	Type 1: Autoimmune destruction of pancreatic beta cells (absolute insulin deficiency). Type 2: Insulin resistance and relative insulin deficiency linked to genetics, obesity, and lifestyle.
SIGNS / SYMPTOMS	• Polyuria (frequent, excessive urination) • Polydipsia (excessive thirst) • Polyphagia (excessive hunger) • Blurred vision and delayed wound healing
TREATMENT / MANAGEMENT	Type 1: Lifelong administration of exogenous insulin. Type 2: Lifestyle modifications (diet, weight loss, exercise), oral antidiabetic agents (e.g., Metformin), and insulin therapy as the disease progresses.